TASHKHEESA

تشخيصة

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Doctor Portal Walkthrough

Everything you need to know to review cases, write reports, and get paid — in 20 minutes of reading.

A practical guide for consulting specialists joining the Tashkheesa platform. Covers onboarding, case flow, clinical tools, payment, and support — in English and Arabic.

Prepared for consulting specialists · **Shifa Hospital · MENA**

tashkheesa.com

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CHAPTER 01

Welcome & What This Is

What Tashkheesa does and where you fit in

Tashkheesa (تشخيصة) is a medical second-opinion platform built for the MENA region. Patients upload their case — scans, labs, prior reports — and receive a written second opinion from a specialist within 24–72 hours. The report is delivered in English or Arabic, depending on what the patient chose.

You are a **consulting specialist**. Your role is focused and bounded: review a case, write a report, done. You do not prescribe routinely, you do not see patients in person, and you are not the primary treating physician. You are the second opinion that helps a patient feel confident about their diagnosis or treatment plan.

What makes this different

Fully async

You work on your schedule. Open a case when you have 30 focused minutes — there are no appointments to keep, no patients waiting on a video call (unless you opt into video consultations when they launch).

Clear scope

Every case comes with the patient's clinical question already written, relevant files already uploaded, and an SLA deadline already set. You are not doing intake.

Privacy first

Patient identity is minimised until you accept. You see initials, specialty, and a one-line concern — enough to decide whether you're the right specialist — before any full case data is revealed.

Hospital-backed

Tashkheesa is integrated with Shifa Hospital Tagamoa (Cairo). If your second opinion recommends hospital-side care, the patient has a direct path from report to treatment.

YOUR ONE SENTENCE

You review the evidence the patient has already gathered, and you write a clear, bilingual report that tells them what you think is going on and what to do next. That's the job.

CHAPTER 02

Getting Started

First login, profile setup, verification

Your first login

1. You'll receive an email from **info@tashkheesa.com** with a link to set your password. Use a browser on a desktop or laptop — the portal is optimised for that, and the first-login setup is easier on a larger screen.
2. Set a password you haven't used elsewhere. Enable your browser's password manager if you have one.
3. Once logged in, you'll see a "Pending Approval" screen if your profile is still being reviewed by the Tashkheesa team. This usually takes under 24 hours on business days.

Completing your profile

Before cases start flowing to you, complete every section of your profile:

PERSONAL INFORMATION

Full name, professional title, languages you can work in (most specialists offer both English and Arabic — this matters because patients choose the language of their report at submission).

PROFESSIONAL CREDENTIALS

Medical degree, fellowship, board certifications, years of experience, current affiliations. These appear on your reports and on your public profile on the Tashkheesa site, so write them the way you'd want them to appear in a newspaper byline.

SPECIALTY AND SUB-SPECIALTY

Pick only the areas where you'd confidently take a case from a family member. The system uses this to match cases to you — if you're a general cardiologist, don't tick "electrophysiology" unless you really mean it.

BIO

A short paragraph that a patient sees before choosing you. Plain-language, warm, no medical jargon. 60–80 words. Mention your special interests.

VERIFICATION

> We'll ask for a scan of your medical license and one credential certificate. These are stored encrypted and only the verification team sees them. After approval, they aren't accessible in your profile — if you need to update them later, contact support.

CHAPTER 03

The Dashboard

Your command centre at a glance

The dashboard is the first thing you see when you log in. It's designed to tell you in under five seconds: what needs your attention, what's about to breach SLA, and how you're doing financially.

The hero strip (top of page)

Four stat tiles tell you the numbers that matter:

Tile	Meaning
Awaiting review	Cases accepted but not yet submitted. Your active workload.
Fast-track due today	Cases with 24-hour SLA that need to close before end of day.
Earnings this month	Paid to you so far this calendar month (in EGP or your local currency).
Completed (30 days)	How many reports you've delivered in the last 30 days.

The card grid (below)

- **Your queue:** up to 6 cases currently awaiting review, newest first
- **Alerts:** system notifications — new cases, SLA warnings, patient messages, reviews
- **This month at a glance:** your average turnaround, patient rating, SLA compliance, reports submitted

> **ONE-CLICK FROM ANYWHERE**
>
> The "Open next case" button in the top right opens the highest-priority case available to you. If you have a Fast-track due today, you'll land on it. If not, the oldest standard case. Always use this if you're unsure where to start.

CHAPTER 04

SLA Tiers Explained

Standard, Fast-track, and Urgent

Every case has a service-level agreement — a promise to the patient about when they'll get their report. You see the SLA on the case from the moment it arrives. Breaching it affects your rating and, for repeat breaches, your ability to accept new cases.

Tier	Deadline	What it means
STANDARD · 72H	72 hours from payment	The default tier. Most cases come in on this SLA. You have three days to open, review, and deliver the report.
FAST-TRACK · 24H	24 hours from payment	Patient paid a premium for a faster response. Treat these as priority. The dashboard highlights them in brass.
URGENT	Under 12 hours	Rare. Usually pre-authorised emergencies. Contact Tashkheesa Care immediately if you cannot meet an Urgent SLA — we'll reassign before the patient notices.

The countdown

On every case, you'll see "Due in Xh Ym". It counts down live. At 25% time remaining, the countdown turns amber. At 10%, it turns red. If you're going to breach, flag it **before** the deadline — reassignment is clean; a breach is not.

IF YOU ACCEPT A CASE YOU CAN'T FINISH

It happens. An emergency comes up, the case turns out to need a sub-specialty you don't cover, whatever. Do not let the SLA breach silently. Message Tashkheesa Care via WhatsApp, briefly say what's happening, and we'll reassign. No judgement, no penalty — as long as you flag it.

CHAPTER 05

The Case Queue

How new cases reach you

The Case Queue is where cases live between "paid and waiting for a doctor" and "in active review." You reach it from the sidebar — it's the second item, right under Dashboard.

Two filter chips at the top

New

Cases that have been assigned to you (or are available in the pool for your specialty) but you haven't opened yet. This is your inbox.

In Review

Cases you've accepted and are actively working on. The draft you've started is saved here, even if you close your browser.

What each row shows

- **Patient initials** (not full name, until you accept) — e.g. "HM, 54 M" for "Hassan Mahmoud, 54, male"
- **SLA chip** (Standard / Fast-track / Urgent)
- **One-line concern** — the patient's own words, no medical jargon
- **Specialty and sub-specialty**
- **File count** (e.g. "7 files")
- **Language** — "AR" or "EN" — the language the final report must be in
- **Due-in countdown** with a coloured status dot

> *Hover over a row for a quick peek at the concern. Click to open the full gated preview.*

CHAPTER 06

Accepting a Case

The gated preview and what it means

When you open a case from the queue, you don't immediately see everything. You see a **gated preview** — enough to decide whether you're the right specialist for this case, but not so much that you've had access to protected patient data if you decide not to take it.

What the preview shows

- Specialty and sub-specialty
- The patient's clinical question (in full, as they wrote it)
- SLA tier and time remaining
- Number and types of files (e.g. "5 files: ECG, Echo, Labs, Stress Test, Chest X-Ray") — *names only, not contents*
- Language of the report to be written

What is locked

- Full patient identity (name, date of birth, contact info)
- File contents (you cannot open any uploaded file)
- Medical history and current medications
- Messaging with the patient
- The report form itself

THE ACCEPT DECISION

If the case fits your specialty and you have time to complete it within the SLA — **click Accept**. Everything unlocks and the countdown starts against your ability to deliver.

If it doesn't fit — close the tab. The case stays in the pool for another specialist. Nothing penalised for not accepting.

Accepting is a commitment

Once you click Accept, the case is assigned to you. Another doctor cannot take it. The patient gets a notification that a specialist is on their case. You are now on the clock.

The only clean way out after accepting is: flag the case to Tashkheesa Care immediately, explain why, and we reassign. A silent missed SLA is different — that hurts the patient, and it hurts your profile.

CHAPTER 07

The Case File

Patient data, uploads, clinical context

Once you accept, the full case file opens. This is where you'll spend most of your time. The layout is two-column: the case data on the left, the report form on the right (with the "Save draft" and "Generate report" buttons sticky at the bottom).

Clinical context panel

Near the top of the case, you'll see three blocks:

- **Question:** exactly what the patient wrote when they submitted. Don't paraphrase — they may use it verbatim when discussing your report with their local doctor.
- **History:** their medical history, as they wrote it. Always plain-patient-language, sometimes terse.
- **Medications:** current meds and doses. Worth cross-checking against their complaint.

Uploaded files

Below clinical context, a list of every file the patient uploaded. Each file has:

- A label (what the patient called it — e.g. "ECG from my GP")
- A detected type (PDF, image, video)
- A small AI-quality badge — "*readable,*" "*blurry,*" "*unsure.*" This is advisory only. Trust your own eyes.

> **THE AI LAYER, HONESTLY EXPLAINED**
>
> When the case is uploaded, an AI classifier organises files into sections (Imaging, Labs, Prior Reports, Other) and flags unreadable scans. It does **not** interpret findings. Think of it as a librarian, not a doctor. Every clinical judgement is yours.

Opening a file

Click the file label. PDFs open in a viewer inside the portal. Images open full-screen with zoom. Videos play inline. You can annotate images (draw on them) — the annotations are saved as part of the case record and optionally attached to your report.

CHAPTER 08

Writing the Report

Findings, Impression, Recommendations

The report is the product. Everything else is scaffolding. Three fields, always in this order:

The three fields

Field	What it is
Findings	What the evidence shows. Objective. Measurements, values, observations. "ECG shows T-wave inversions in V4–V6. Echocardiogram: LV ejection fraction 55%, mild diastolic dysfunction." This section describes — it does not interpret.
Impression	Your clinical interpretation. What do you think is going on? This is where your expertise is valuable. Be direct, be bounded by uncertainty where it exists. "Likely early ischemic cardiomyopathy. T-wave changes and exertional symptoms warrant further evaluation."
Recommendations	What should happen next. Concrete, actionable, ideally in the patient's home country. "Recommend stress echocardiogram and referral to interventional cardiology. Continue current medications. Follow-up in 2 weeks."

Writing for your audience

Your primary audience is **the patient's local treating doctor**. They need to understand what you found, what you think, and what you recommend — with enough detail to act, but not so much detail that your report becomes a textbook. Aim for what you'd want if you were receiving this report from a colleague.

Your secondary audience is **the patient**. They will read this report. Avoid acronyms the first time you use them. Don't soften findings — but frame them clearly.

Save draft vs Generate report

Save draft

Stores your current text. Does nothing else. You can come back and finish later. Drafts autosave every 30 seconds, but clicking Save explicitly is safer.

Generate report

Finalises the report as a PDF, sends it to the patient in their chosen language, and locks the case. This action is irreversible. Once clicked,

the patient has access and the case is considered complete.

Bilingual reports

If the patient chose Arabic at submission, you write in Arabic (or dictate in English and our translation team finalises — you'll see that option on the case). The portal displays the language expectation clearly on every case.

EXAMPLE FINDINGS (EN)

ECG demonstrates T-wave inversions in leads V4 through V6, consistent with lateral ischemia. Echocardiogram shows preserved left ventricular ejection fraction at 55 percent, with mild diastolic dysfunction. Troponin I was negative at presentation.

مثال على النتائج (AR)

يظهر تخطيط القلب الكهربائي انقلاباً في الموجة T في المشتقات V4 حتى V6، بما يتوافق مع نقص تروية جانبي. يُظهر الإيكو أن جزء القذف البطيني الأيسر محفوظ عند 55 بالمئة، مع خلل انبساطي طفيف. كان تروبونين I سلبياً عند التقديم.

WHAT MAKES A TASHKHEESA REPORT GOOD

Three qualities, in order: **clear** (a 60-year-old patient can read it), **actionable** (their doctor knows exactly what to do next), **humble** (you state uncertainty where it exists rather than hiding it).

CHAPTER 09

Requesting Files & Flagging Errors

When the case isn't complete

Sometimes the patient's upload is missing something critical, or a file is unreadable, or there's an inconsistency in the story. You have two tools for this, both on the case page.

Request additional files

Under the uploaded files section, you'll find "Request additional files." Click it, describe what you need (e.g. "Lateral chest X-ray from the same imaging session" or "The original ECG report, not a photo of the printout"), and send.

The patient gets a notification, uploads the additional file, and the SLA clock pauses while they do. Once uploaded, the clock resumes.

> *Be specific about what you need. "More history" is not useful. "Confirmation of whether the episodes of chest pain occurred at rest or with exertion" is useful.*

Flag an error

If the case has a critical issue — wrong specialty, wrong patient age category (paediatric case routed to adult cardiology, etc.), obviously fake or fraudulent files — use "Flag case to Care" rather than trying to resolve it yourself. You can do this from the "•••" menu at the top-right of the case.

> **DO NOT DIAGNOSE OFF-LABEL**
>
> If the case is outside your specialty, flag it — don't write a report anyway. A generalist cardiology case being answered by an electrophysiologist who isn't confident in general cardiology helps no one.

CHAPTER 10

Video Appointments

Live consultations — coming soon

HEADS UP — NOT LIVE YET

Video consultations will launch in a future release. You'll see an "Appointments" item in the sidebar marked "Coming soon." This chapter documents the flow so you know what to expect when it's enabled. None of this is active on your account today.

When it launches

Patients will be able to book a 20-minute follow-up video call after receiving your written report. They pay an additional fee, and you're compensated separately for the call. The call is scheduled around your availability, not theirs.

What you'll need

- A quiet room, good lighting, camera at eye level
- Wired internet preferred, or strong Wi-Fi
- The case file open in a second tab during the call
- 20–25 minutes of uninterrupted time

Setting your availability

When the feature launches, you'll set weekly availability blocks (e.g. Tuesdays and Thursdays, 7pm–9pm). Patients book inside those blocks. You can edit availability any time, but try to give at least 48 hours' notice before cancelling an open slot.

CHAPTER 11

Patient Messaging

Secure, case-scoped communication

Once you accept a case, a messaging thread opens between you and the patient. It lives on the case page itself — no separate inbox to manage.

What you should use it for

- Clarifying a specific clinical detail (e.g. "The pain — is it worse at night, or just after meals?")
- Confirming receipt of a requested additional file
- Letting them know your report will be ready shortly (reassurance)

What you should not use it for

- Delivering the actual opinion — that goes in the report
- Booking a private consultation or sharing personal contact details
- Debating clinical decisions with the patient's primary doctor (if they try to insert themselves into the thread)

> **LANGUAGE**
>
> Message in whatever language the patient used — the thread auto-detects. If a patient writes in Arabic and you reply in English, a translation prompt appears to help.

CHAPTER 12

Prescriptions

When you can issue one

Tashkheesa is not a prescribing platform in the routine sense. Your job is to offer a second opinion, not to manage ongoing treatment. That said, there are narrow situations where a prescription from you is appropriate.

When a prescription is OK

- You clearly identify a missed medication in the patient's regimen (e.g. statin indicated but not prescribed) and you recommend adding it with appropriate dosing
- You identify a clear dosing error in their current medication that needs correcting
- The patient's local system will honour a second-opinion prescription (varies by country)

When a prescription is not OK

- Controlled substances of any kind
- Anything requiring physical examination (antibiotics for undiagnosed infections, etc.)
- Weight loss drugs, erectile dysfunction meds, hair loss meds, or anything routinely sought online — Tashkheesa is not that platform

DEFAULT ANSWER: RECOMMEND IN THE REPORT, DON'T PRESCRIBE

For almost every case, writing "Recommend starting atorvastatin 20mg daily" in your Recommendations section is sufficient. The patient's local doctor issues the actual prescription. Issue prescriptions through the portal only when there's a clear reason the patient cannot get it locally.

CHAPTER 13

Your Earnings

How you get paid, how much, when

The fee structure

Tashkheesa charges the patient a fee per case, depending on specialty and SLA tier. Of that fee, **20%** goes to you as the reviewing specialist. The platform keeps 80% to cover hospital partnership, operations, translation, technology, and marketing.

A worked example

Line	Amount (EGP)
Patient pays (Fast-track cardiology review)	**1,500**
Your fee (20%)	**300**
Platform fee (80%)	1,200

Payouts

- **When:** Monthly. Payouts run on the 1st of each month for the previous month's completed cases.
- **How:** Direct bank transfer (EGP), Stripe Connect (international), or Instapay (Egypt only — faster).
- **Minimum:** No minimum. If you completed one case last month, you get paid for one case.
- **Statement:** You'll receive an itemised statement every month showing each case paid out, and what was deducted (rare — only for refunds).

Refunds

If a patient requests a refund and it's granted (for example, your report was clearly off-target or the case shouldn't have been accepted), the corresponding doctor fee is reversed out of the next month's payout. This is rare — Tashkheesa Care reviews every refund request before approving.

> **TAXES**
>
> You are responsible for declaring and paying any tax on your Tashkheesa earnings. We issue a year-end summary you can share with your accountant. If you need specific tax documentation for your jurisdiction, email **finance@tashkheesa.com**.

CHAPTER 14

Reviews & Your Reputation

Patient feedback loop

After the patient receives your report, they can leave a rating (1–5 stars) and an optional comment. This is visible on your public profile on the Tashkheesa site and on internal analytics.

What patients rate on

- **Clarity:** did the report make sense?
- **Actionability:** did they know what to do next?
- **Responsiveness:** did you reply to messages promptly?
- **Overall trust:** would they recommend you?

Responding to reviews

You can reply publicly to any review — we recommend doing so for 5-star reviews (short, warm thank-you) and for 3-star-or-below reviews (acknowledge, offer to clarify). For genuinely unfair reviews, flag them to Care — we review and remove if they violate our guidelines.

A NOTE ON RATINGS

Medical platforms skew high — 4.7+ is typical. Consistent 5-star requires genuine warmth and clarity. If you're below 4.5, look at the comments, not the number — there's usually something specific patients are asking for (more detail, faster response, clearer language). That's actionable.

CHAPTER 15

Analytics

Your performance over time

The Analytics page lives under "Insights" in the sidebar. It shows you, as a doctor, the numbers the Tashkheesa team also sees.

The four hero stats

- **This month's earnings** — real-time, in EGP
- **Cases completed** — rolling 30 days
- **Average turnaround** — hours from accept to submit, rolling 30 days
- **Patient rating** — rolling 90 days

Charts

- Earnings over the last 6 months (line chart)
- Cases by complexity — simple / moderate / complex (bar chart)
- SLA compliance — percent of cases delivered within SLA (gauge)

> *The analytics page updates nightly. If you submitted a case five minutes ago, it's already in your completed count but not yet in monthly earnings — that rolls up at midnight.*

CHAPTER 16

Alerts & Notifications

How we keep you in the loop

Alerts are the signal layer of the portal. They tell you about things that need your attention without you having to go looking for them.

Where alerts appear

- On the Dashboard, in the "Recent Alerts" card
- On the Alerts page (sidebar), with a full history
- By email (for critical ones only — new case, SLA warning, report delivered)
- By WhatsApp (optional — opt in from your Profile page)

Types of alert

Icon	Type	Meaning
📩	New case	A case has been assigned to you or added to your specialty pool
⚠️	SLA warning	A case you've accepted is approaching deadline
💬	Patient message	A patient has messaged you on an active case
✅	Report delivered	Confirmation your report was received and processed
⭐	Review received	A patient left you a rating
💰	Payout sent	Your monthly payout has been processed

Muting alerts

From your Profile page, you can mute specific alert types (for example, turn off email alerts for patient messages but keep WhatsApp). You cannot mute SLA warnings — they're always on, by design.

CHAPTER 17

Support & Escalation

Who to contact, when

The Tashkheesa Care team is real people, reachable in real time, specifically trained to help doctors on the platform. Never feel like you're bothering us — a flagged case handled in 5 minutes is better than a silent SLA breach.

CLINICAL OR CASE QUESTION

WhatsApp: +20 10 XXXX XXXX

Response within 2 hours, 8am–11pm Cairo time

TECHNICAL ISSUE WITH PORTAL

support@tashkheesa.com

Response within 4 hours on business days

PAYMENT OR EARNINGS QUESTION

finance@tashkheesa.com

Response within 24 hours, Sun–Thu

EMERGENCY SLA BREACH RISK

WhatsApp: +20 10 XXXX XXXX (same line)

Flag as URGENT, response within 30 minutes

What to include when you contact us

- Your doctor name
- The case reference number (visible on every case — e.g. TSH-2026-1205)
- A one-line summary of what's happening
- Screenshot if it's a technical issue

WELCOME TO TASHKHEESA

Thank you for joining the platform. The work you do — giving patients clarity, second opinions, a real path forward — is genuinely valued. If anything in this guide was unclear or anything on the portal doesn't behave the way it's described here, please tell us. We update this guide monthly based on what doctors actually need.

Questions? **info@tashkheesa.com**